

EOPE Adjudication — Working Instruction

How to adjudicate an early-onset preeclampsia (EOPE) case — PROTECT-Africa (Protocol A202501)

Study	PROTECT-Africa (EOPE)	Applies to	All EOPE adjudicators
Governing docs	OAC Charter v2.0; SOP-ADJ-001 to 004	Forms	FORM-ADJ-01 to 14 (Forms Pack)
Diagnostic basis	PROGNOSIS-aligned / ACOG population definitions; protocol-precedence applies	Classification	Confidential — governance use

1 Purpose and golden rules

This instruction tells you, as an EOPE adjudicator, exactly what to do with a case from the moment it reaches your review area to final classification. You provide an independent, blinded, reproducible endpoint classification against a single clinical reference standard. You classify outcomes only — you do not direct clinical care, assess safety causality, or replace SAE/SUSAR/SOC reporting.

Five golden rules that override convenience:

- **Stay blinded.** You never see, request, infer or use sFlt-1/PlGF, sEng, or any point-of-care result until database lock.
- **Review independently.** You do not confer with the other reviewer and cannot see their entry before concordance is determined.
- **Protocol wins.** Where a protocol definition differs from a general standard, the PROTECT-Africa definition prevails.
- **Never guess.** If data are missing, ambiguous or inconsistent, raise a query or mark the attribute not adjudicable — do not pick the more likely answer.
- **Document the reason.** Every classification carries a written rationale naming the criteria applied and the data relied on.

2 Before you start each case

Step 1. Confirm the package is releasable. Only open packages the Coordinator has marked “blinding check passed” on FORM-ADJ-01. If you spot any biomarker/POC value or unblinding free-text, STOP, do not classify, and report it (FORM-ADJ-09).

Step 2. Check for a conflict. If you are an investigator at, or provide clinical care for, the case’s site, you must recuse. Record the recusal (FORM-ADJ-08) and do not adjudicate that case.

Step 3. Confirm it is an EOPE case and identify the window. PROTECT-Africa uses rule-out within 1 and 2 weeks and rule-in within 2 and 4 weeks of Visit 1. On FORM-ADJ-02 you will tick either the 1/2-week or the 2/4-week window. Never apply the LOPE 14-day composite.

3 Reviewing the case narrative

Read the whole standardized package: clinical history; serial blood pressure and proteinuria; laboratory results; obstetric course and interventions; delivery details; and neonatal outcomes. Establish the gestational-age timeline

first, because onset timing drives the EOPE classification. Use the protocol dating hierarchy, preferring an ultrasound performed at or before 22 weeks where available.

Adjudicate the full case, including uneventful ones. An outcome-free evaluable case is still classified (“No PE” with rationale). Skipping quiet cases biases the analysis denominator.

4 Completing FORM-ADJ-02 (your classification)

Record each attribute below with its rationale. Apply the diagnostic definitions in Section 8.

Attribute	What to record	EOPE-specific rule / threshold
Primary outcome	No PE / PE / Severe PE / Eclampsia / HELLP	HELLP: AST >70 IU/L, LDH >600 IU/L, platelets <100,000/ μ L
Timing of onset	Date and gestational age at which the case FIRST meets diagnostic criteria	By onset, not by delivery date
Onset classification	Early-onset vs late-onset	Onset <34+0 wk = early; $\geq$ 34+0 wk = late (see Section 5)
Severe features	Present: Yes / No	SBP $\geq$ 160 and/or DBP $\geq$ 110; severe-range proteinuria; renal/liver impairment; pulmonary oedema; neuro/visual; low platelets/haemolysis; IUGR
Maternal composite	Tick items within the window, or None	Maternal death; pulmonary oedema; acute renal failure; cerebral haemorrhage; cerebral thrombosis; DIC
Neonatal composite	Tick items within the window, or None	Perinatal/fetal death; delivery <34 wk; IUGR; placental abruption; respiratory distress; NEC; IVH
Relationship to PE	Attributable / Not attributable / Uncertain	Clinical-classification judgement (not safety causality)
Adjudicable	Yes / Not adjudicable (with reason)	If not, route per Section 6

Then write your rationale: for each classified attribute, name the criteria applied and the specific data points relied on. Sign and date. Do this without conferring with the other reviewer.

5 The 34+0-week boundary rule (EOPE-critical)

Classify by gestational age at which the case first meets diagnostic criteria for PE/eclampsia/HELLP — not by delivery date. Onset before 34+0 weeks is early-onset; at or after 34+0 weeks is late-onset.

Do not decide a boundary case alone. Any case near 34+0 weeks requires at least three OAC members or a quorate panel discussion, with the rationale documented. Give your provisional view, then flag for escalation.

6 Decision, escalation and data-handling rules

Two concordant reviews → final. If your classification matches the second independent reviewer, it is accepted as final and recorded in the EOPE adjudication log (FORM-ADJ-11).

Discordant → third review. A third independent, non-conflicted reviewer adjudicates on FORM-ADJ-04; the case is resolved by majority of reviewing members at the next scheduled or expedited meeting.

No majority → panel / Chair tie-break. The quorate OAC discusses; on a documented persistent deadlock the Chair casts the deciding view. Any dissent is recorded.

Missing / unclear data → query. Raise a written data query (FORM-ADJ-05); hold that attribute pending the site response.

Still unresolved → not adjudicable. If no classification is possible within the protocol window, record the attribute as “not adjudicable” with a reason (FORM-ADJ-06). Never drop it silently.

7 Expedited (sentinel) path

Sentinel events are pulled out of the normal batch and adjudicated immediately on FORM-ADJ-07: maternal death, eclampsia, HELLP, stroke/cerebral haemorrhage, pulmonary oedema, acute renal failure, DIC, ICU admission, stillbirth, neonatal/perinatal death. Expedited review is for endpoint classification only; it never replaces or delays SAE recording or safety notification.

8 Diagnostic criteria to apply (EOPE reference)

Protocol-precedence applies; thresholds below are the EOPE operational definitions.

Term	Operational definition
Hypertension	SBP ≥ 140 and/or DBP ≥ 90 mmHg on two occasions ≥ 6 hours apart (within 1 week), or controlled pre-existing hypertension on therapy.
Chronic hypertension	Diagnosed pre-conceptionally or before 20 weeks, persisting >12 weeks postpartum.
Proteinuria	≥ 0.3 g/24 h (or dipstick $\geq 2+$, or spot protein/creatinine ratio at the protocol threshold).
Preeclampsia (PE)	New-onset hypertension and proteinuria after 20 weeks (or hypertension with other maternal organ or uteroplacental dysfunction).
Severe preeclampsia	PE with ≥ 1 severe feature: SBP ≥ 160 and/or DBP ≥ 110 ; severe-range proteinuria; impaired renal function; pulmonary oedema; impaired liver function with RUQ/epigastric pain; neurological/visual symptoms; thrombocytopenia/haemolysis; IUGR.
Eclampsia	New-onset tonic-clonic seizures in a woman with PE, not attributable to another cause.
HELLP syndrome	Haemolysis, elevated liver enzymes, low platelets — EOPE: AST >70 IU/L; LDH >600 IU/L; platelets $<100,000/\mu\text{L}$.
Superimposed PE	Chronic hypertension with new-onset proteinuria after 20 weeks, or a sudden increase in proteinuria/BP, or signs of severe PE.
Acute renal failure (AKI)	Rise in serum creatinine (protocol threshold, e.g. ≥ 1.1 mg/dL or doubling of baseline) and/or oliguria <500 mL/24 h, absent pre-existing CKD; graded per KDIGO.
Pulmonary oedema	Abnormal pulmonary fluid accumulation with impaired gas exchange, clinically and radiographically confirmed.
Stroke / cerebral haemorrhage	Intracranial haemorrhage or ischaemic cerebrovascular event confirmed by neuroimaging (CT/MRI).
Cerebral thrombosis	Neurologically symptomatic ischaemic event from a cerebral arterial thrombus, confirmed by neuroimaging (EOPE composite).

9 Form quick-map

Step / situation	Form
Package release & blinding check (Coordinator)	FORM-ADJ-01
Your independent EOPE classification	FORM-ADJ-02
Third (tie-break) review of a discordant case	FORM-ADJ-04
Missing / ambiguous data query	FORM-ADJ-05
Attribute cannot be classified	FORM-ADJ-06
Sentinel event (out-of-cycle)	FORM-ADJ-07
Conflict declaration & recusal	FORM-ADJ-08
Suspected/actual unblinding	FORM-ADJ-09
Final classification register	FORM-ADJ-11
Pre-transfer QC & Chair sign-off	FORM-ADJ-12
Meeting minutes (decisions, dissent, recusals)	FORM-ADJ-13

